per cent believes that it is unsafe for an unaccompanied woman to be outside the house. Only 13 per cent of Mumbai believes that an unaccompanied woman is safe outside the house at any time of the day, and for New Delhi this drops to 2 per cent. [1]

In India, teacher absenteeism is at 23.6 per cent [2] and the absence rate for medical personnel is 40 per cent. [3] Immunization rates are at about 60 per cent. [4]

The expenditure per student in government schools rose from Rs 2455 in 2010 to Rs 4385 in 2016. Learning outcomes *declined* in this period. In 2010, 50.7 per cent of children in class 5 in government schools could read a class 2 level text, but this declined to 42.2 per cent in 2014. Parents voted with their feet, and switched from free government schools to private schools charging a fee. Enrolment in government schools fell by 11.1 million while it rose by 16 million in private schools, from 2010 to 2016. [5]

At the end of many decades of very large expenditures in health and education in India, we are holding dismal outcomes. This shows the difficulties of state intervention.

'Public economics' is the idealized world of economic analysis, where market failure is identified. This leads to policy analysis, about how state interventions can be undertaken in order to address market failure. These are implemented through organizations, and their design and management lies in the field of 'public administration'. In an ideal world, we would detect market failure using public economics, and then do public administration in order to intervene in society so as to address the market failure.

In the real world, many interventions are ill-advised: they are not motivated by the desire to address market failure. And many interventions fail. The state is supposed to be a machine that converts coercive power into human welfare. But human societies fare poorly in controlling and channelling the monopoly of violence into welfare and justice.